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PPOM I: Lecture # 81

Anxiety Disorders:

General Anxiety Disorder, Panic Disorder, Social Anxiety Disorder

Principles and Practice of Osteopathic Medicine I

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Session Objectives

At the conclusion of the session/course, the student should be able to:

1. Identify patients with anxiety by utilizing the DSM-5 criteria to differentiate between:
 - general anxiety disorder
 - panic disorder
 - social anxiety disorder
2. Recall epidemiologic information related to anxiety disorders to develop a framework for identifying those at risk of these disorders
3. Create a differential diagnosis of anxiety disorders including psychiatric and medical causes
4. Develop a treatment plan for patients diagnosed with an anxiety disorder
5. Identify risk factors that may affect outcomes in patients with anxiety disorders

Why should we talk about Anxiety Disorders (AD)?

- AD commonly occurring mental health conditions.
- Anxiety disorders are often unrecognized in primary care settings and years-long delays in treatment initiation occur.
- Anxiety can be a chronic condition characterized by periods of remission and recurrence.
- Prevalence in adults was 26.4% for men and 40.4% for women.
- <https://www.uspreventiveservicestaskforce.org/uspstf/draft-recommendation/anxiety-adults-screening>

Normal:

Universal human experience

Can be beneficial in many events

Pathological when the following criteria are affected:

Excessive

Intensity

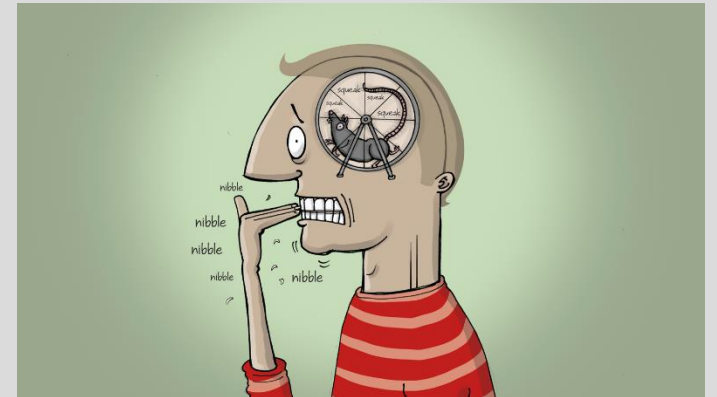
Chronicity

Impairment

Any worry or anxiety that negatively affects an individual's ability to perform daily tasks should be investigated as a possible anxiety disorder.

Any worry or anxiety that negatively affects an individual's ability to perform daily tasks should be investigated as a possible anxiety disorder.

Anxiety is manifest in physical, affective, cognitive, and behavioral domains.



Common findings among Anxiety Disorders

Source: Anxiety Disorders. Massachusetts General Hospital Comprehensive Clinical Psychiatry, Chapter 32, 353-366.2016

Physical

- Increased autonomic response

Affective/ Emotional

- Range of feelings of fear/uneasiness, edginess to terror and panic

Cognitive

- Worry, apprehension, and train of thoughts related to real or perceived threats, catastrophic thinking

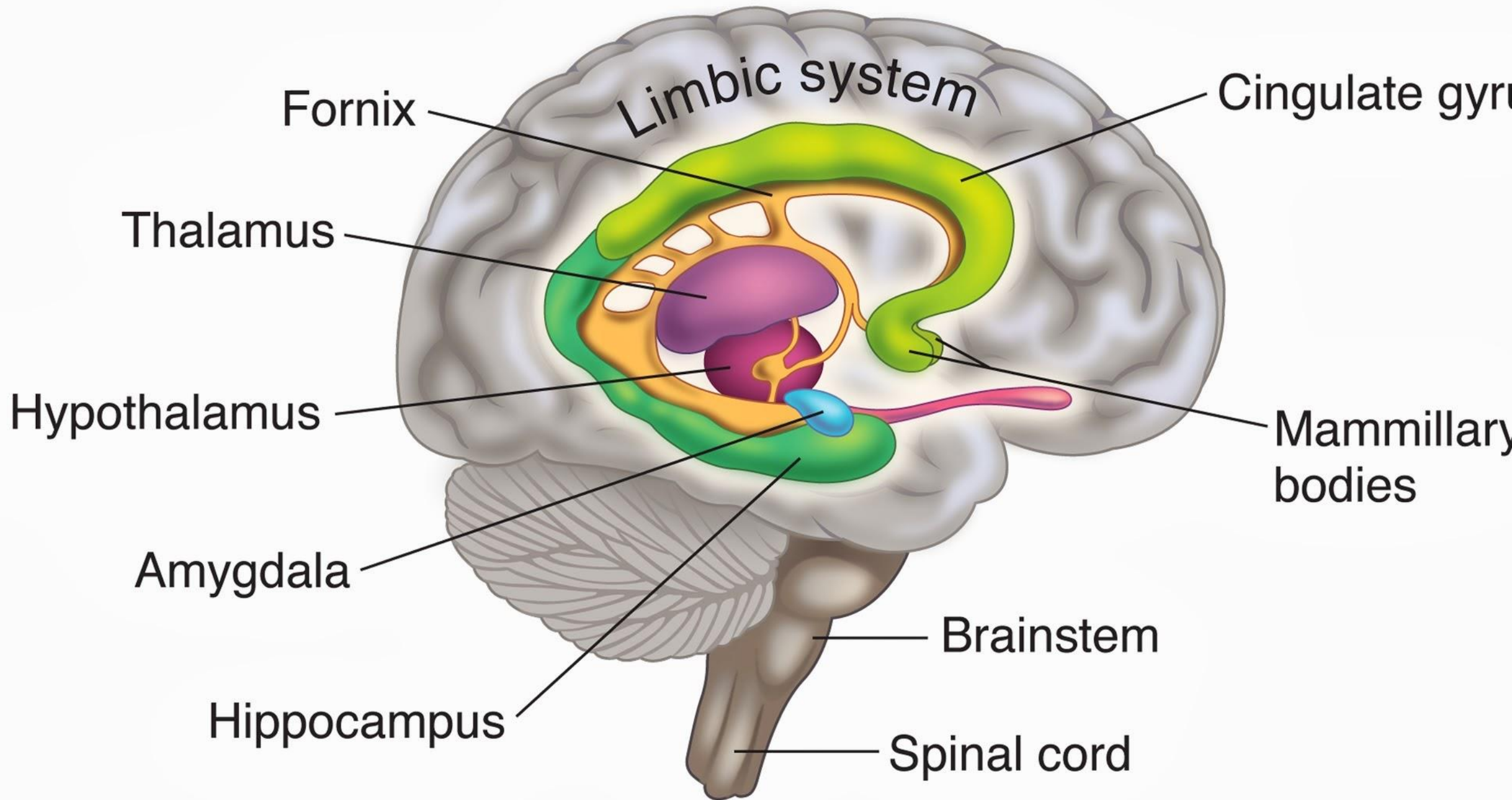
Behavioral

- Responses such as avoidance, escape, and safety-seeking behaviors

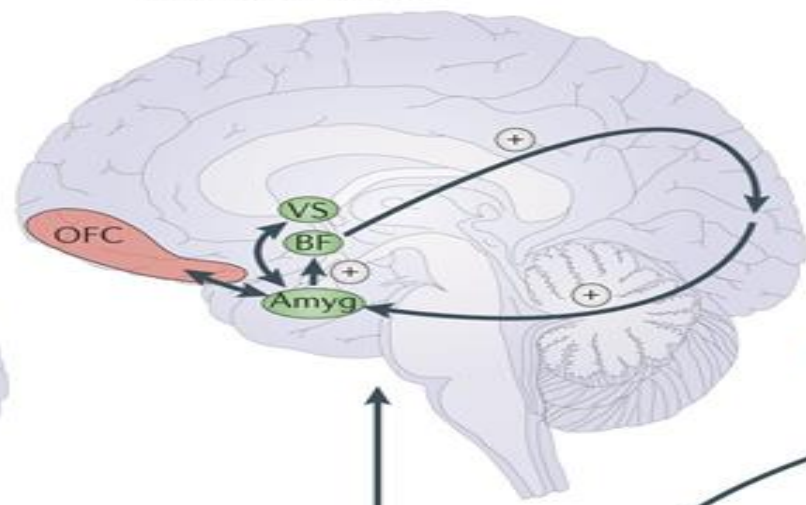
Most common factors that lead to AD:

- genetics,
- brain chemistry,
- personality,
- life events,
- adverse childhood experiences (ACEs)* are stressful or traumatic events (such as physical, sexual, or emotional abuse or neglect) that occur during childhood,
- In most AD, dysregulation and low levels of γ -aminobutyric acid (GABA) and serotonin likely play a role,
- The amygdala- a portion of the middle brain that processes emotion, memory, and fear, is a key area of interest on brain-imaging studies related to anxiety disorders.

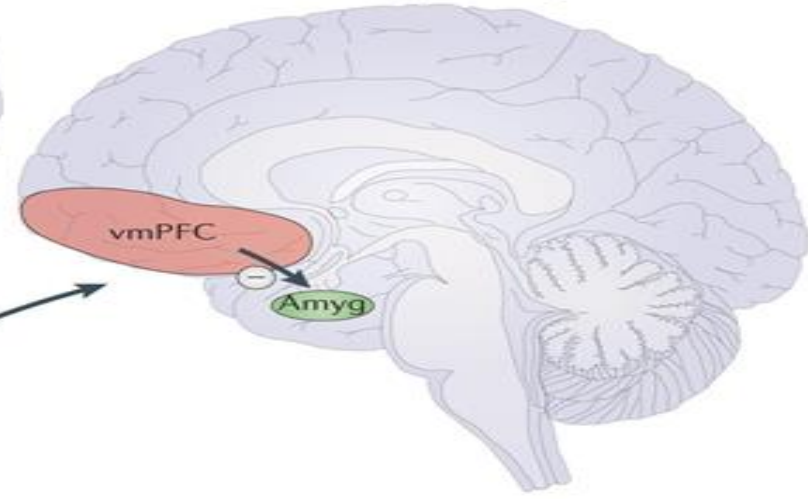
**Roughly 40% of people in the United States report experiencing two or more ACEs.*



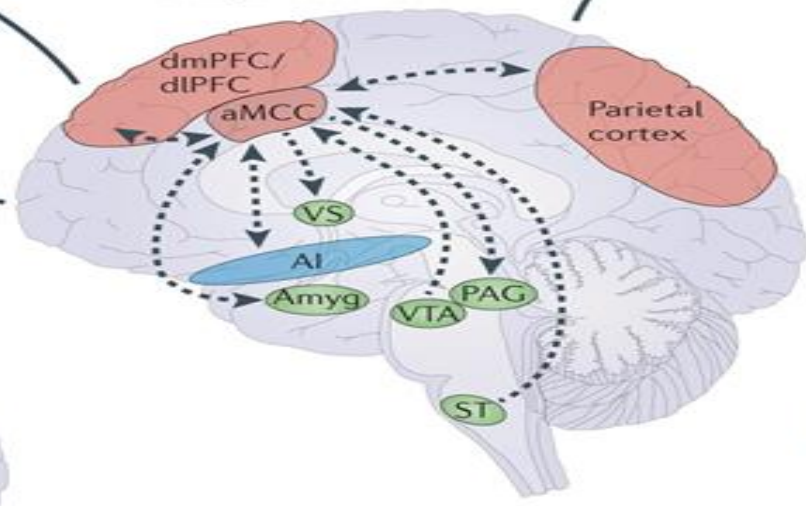
b Increased threat attention and hypervigilance



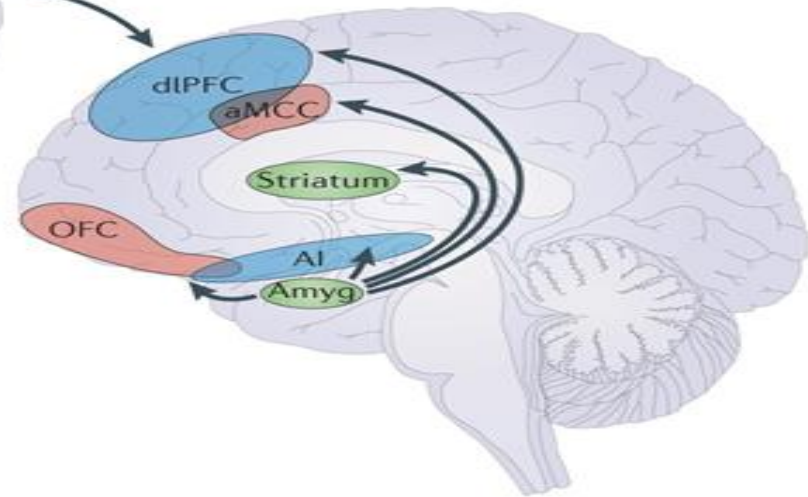
c Deficient safety learning



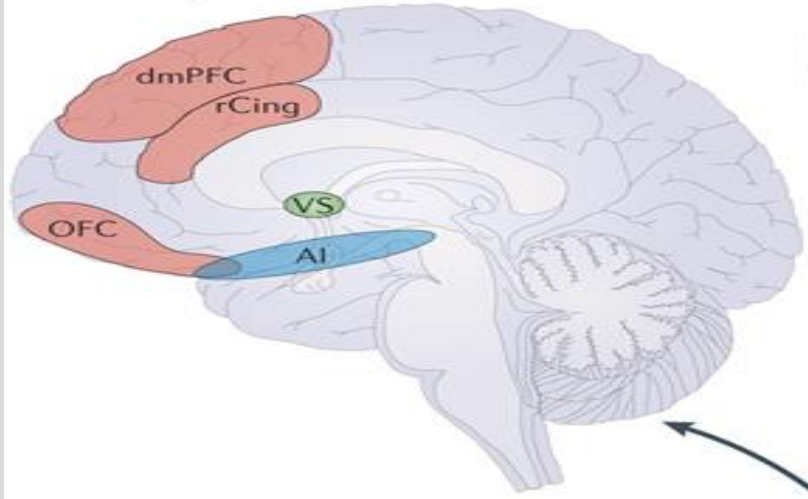
f Maladaptive control



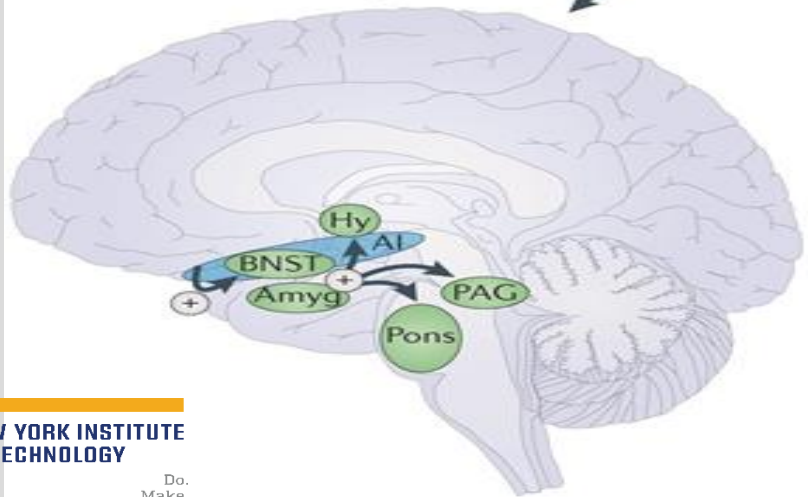
d Behavioural and cognitive avoidance



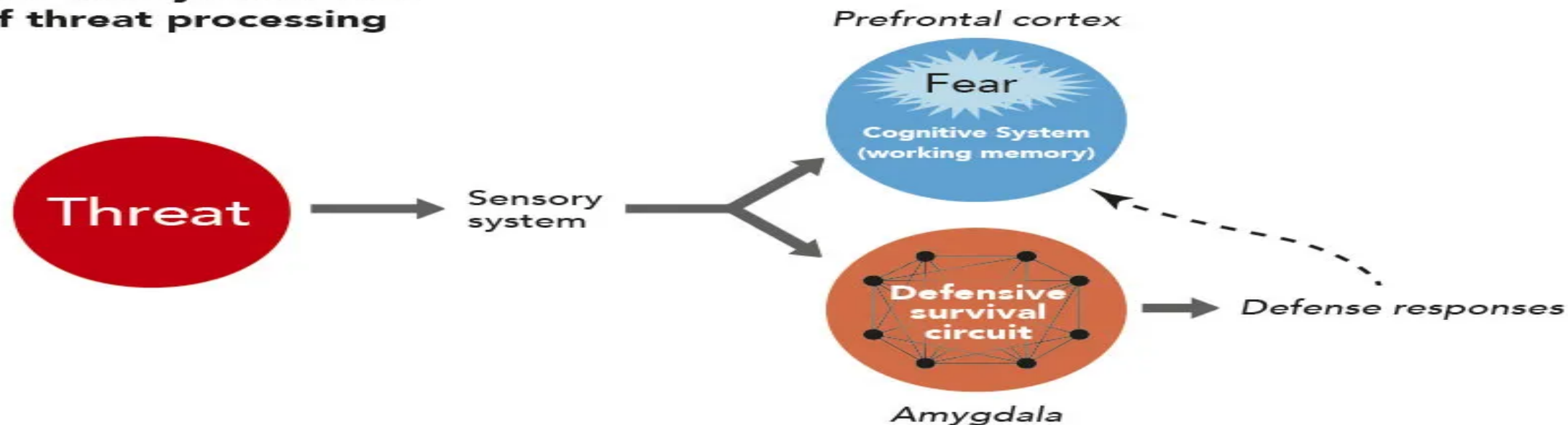
a Inflated estimates of threat cost and probability



e Heightened reactivity to threat uncertainty



The two-systems view of threat processing



Cognitive processes contributing to the conscious experience of fear



Research on anxiety disorders in under represented groups

Source:

1. Ching, T. H. W. (2021). Culturally Attuned Behavior Therapy for Anxiety and Depression in Asian Americans: Addressing Racial Microaggressions and Deconstructing the Model Minority Myth. *Cognitive and Behavioral Practice*.
2. Laura J. Samander, & Jeffrey Harman. (2022). Disparities in Offered Anxiety Treatments Among Minorities. *Journal of Primary Care & Community Health*, 13.
3. Chodzen, G., Hidalgo, M. A., Chen, D., & Garofalo, R. (2019). Minority Stress Factors Associated With Depression and Anxiety Among Transgender and Gender-Nonconforming Youth. *Journal of Adolescent Health*, 64(4), 467–471.

DO THIS:

- Know your blind spots (your beliefs about eating disorders, substance use/dependence, etc)
- Catch your microaggressions,
- Discuss mental illness as a biological disorder rather than a personalized issue,
- Work hard to gain your patient's trust: Know that people from underrepresented groups may have experience marginalization, including in healthcare.

Source: <https://www.nimh.nih.gov/about/organization/dar/research-tools>

Minority stress model for mental health:

- a. Stigma and discrimination frameworks and models are critical for understanding, measuring, and intervening to reduce stigma and discrimination and improve outcomes.
- b. Clear frameworks and models about how stigma and discrimination may occur are important to develop anti-stigma or anti-discrimination interventions.
- c. Multiple factors need to be considered to decide which framework or model is a good fit for a particular research or service project.
- d. The frameworks and models listed below may help guide measurement, research, intervention development, and policy efforts.

DO NOT DO THIS

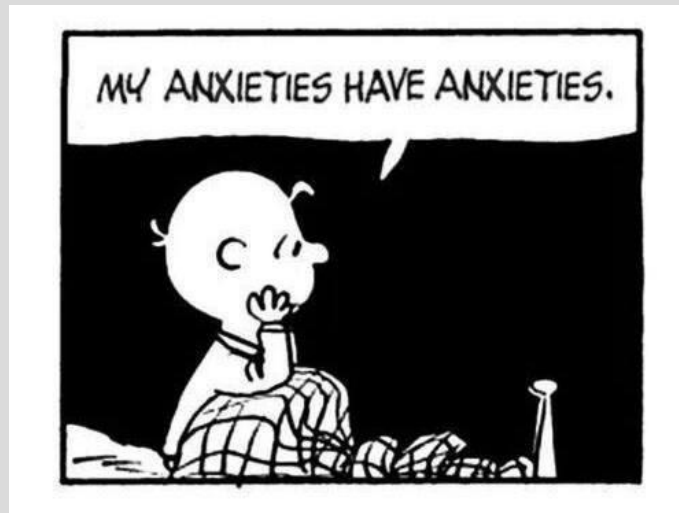
“You just need to try harder...”



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Generalized Anxiety Disorder (GAD)



Generalized Anxiety Disorder (GAD)

Source: Anxiety Disorders. Massachusetts General Hospital
Comprehensive Clinical Psychiatry, Chapter 32, 353-366.2016

Symptoms associated with GAD: **WATCHERS**

- a. Worry,
- b. Anxiety,
- c. Tension in muscles,
- d. Concentration difficulty,
- e. Hyperarousal (or irritability),
- f. Energy loss,
- g. Restlessness,
- h. Sleep disturbance

Worry and tension about a variety of events and activities in one's daily life that are:

Persistent,
Excessive,
Difficult-to-control.



Epidemiology of GAD

Female>>Male

Unmarried/minority, low SES>>

Onset at late teens to early 20s, but may occur at any age



Sex:
2:1 F:M



- Higher risk among: unmarried, minority populations, lower socioeconomic status groups
- Up to 80% have other psychiatric disorder including depression
- Also seen in patients with unexplained poly-organ symptoms

Diagnostic Criteria for GAD

BOX 32-4 DSM-5 Diagnostic Criteria: Generalized Anxiety Disorder (300.02 (F41.1))

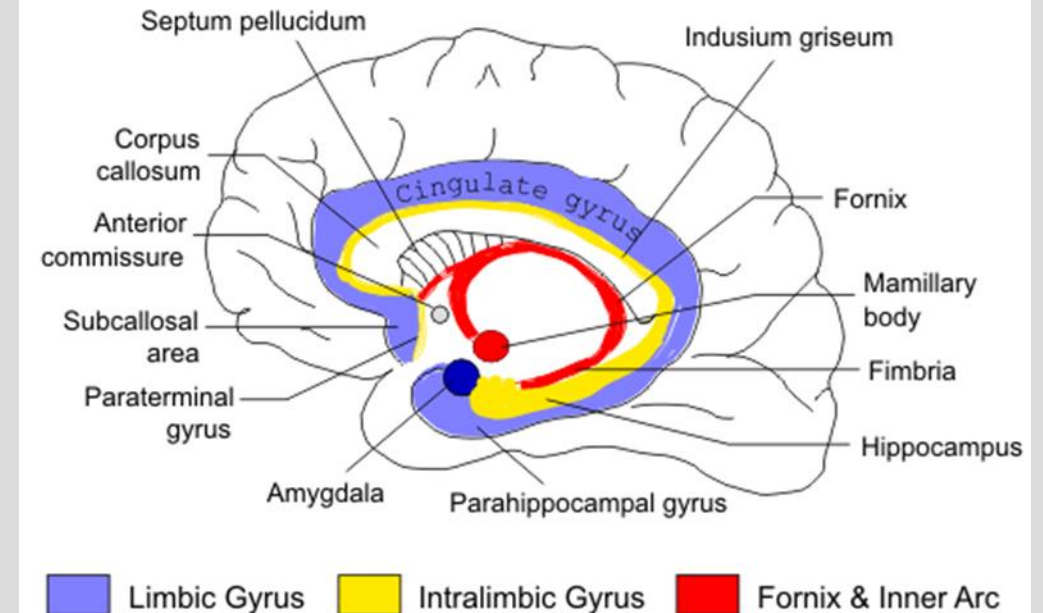
- A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).
- B. The individual finds it difficult to control the worry.
- C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months):

Note: Only one item is required in children.

- 1. Restlessness or feeling keyed up or on edge
 - 2. Being easily fatigued
 - 3. Difficulty concentrating or mind going blank
 - 4. Irritability
 - 5. Muscle tension
 - 6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).
- D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.
 - E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).
 - F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

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The Limbic System



DSM-V Diagnostic Criteria for GAD

THE DIAGNOSTIC AND STATISTICAL MANUAL OF MENTAL DISORDERS , ED 5, (COPYRIGHT 2013).
AMERICAN PSYCHIATRIC ASSOCIATION.

DSM5 Diagnostic Criteria

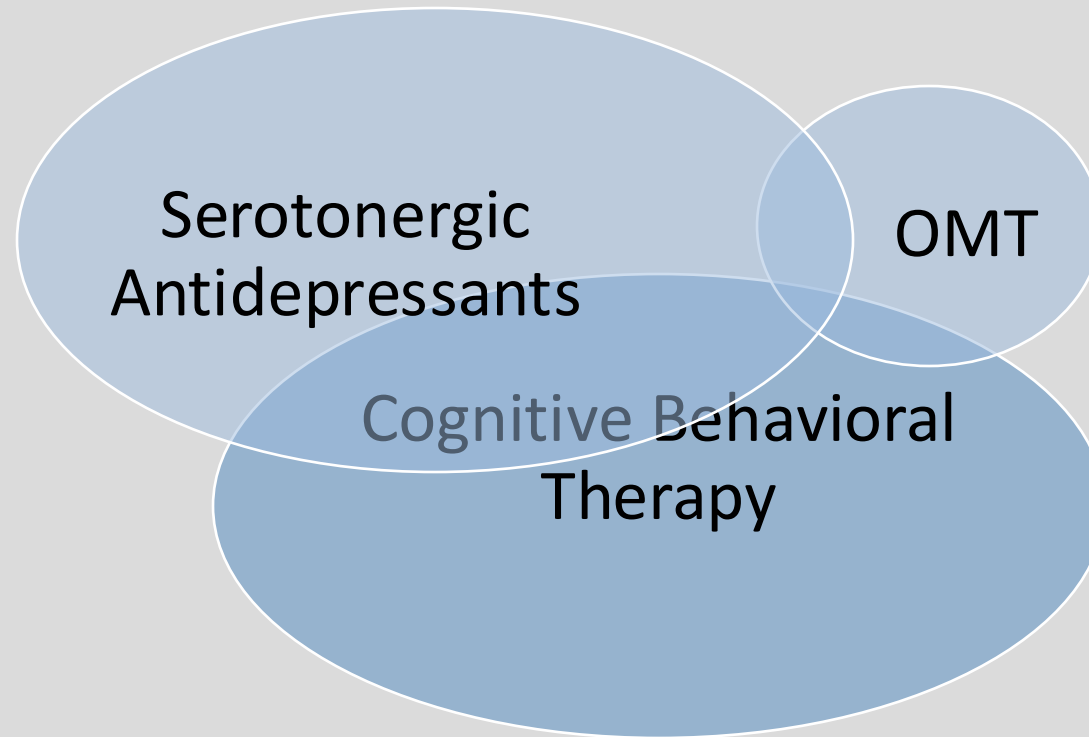
- **At least 6 months**, more days than not with:
- Associated with **3 or more** of the following:
 - Restlessness or feeling on edge
 - Being easily fatigued
 - Difficulty concentrating or mind going blank
 - Irritability
 - *Muscle tension*
 - Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep)
- **Causes distress or impairment in social, occupational or other life functions**
- **Not attributed to substance use or medical causes**
- **Is not explained by another mental disorder**

Treatment Approach to Generalized Anxiety Disorder

Hunot V, Churchill R, Silva de Lima M, Teixeira V. Psychological therapies for generalised anxiety disorder. Cochrane Database Syst Rev 2007

Academic Journal

By Reinecke, Andrea; Hoyer, Jürgen; Rinck, Mike; Becker, Eni S.. In *Journal of Behavior Therapy and Experimental Psychiatry*. March 2013 44(1):1-6.



CBT treatment for GAD

National Institute for Health and Clinical Excellence, 2007. National Institute for Health and Clinical Excellence : Nice guideline 22 – Anxiety: Management of anxiety (panic disorder, with or without agoraphobia, and generalised anxiety disorder) in adults in primary, secondary and community care.

- CBT approaches are recommended as first-line interventions in GAD
- experience of habituation to fear stimuli during repeated exposure or relaxation
- effective in reducing attentional bias to threat
- Other options (after first line options and CBT failure):
 - Benzodiazepines
 - Lorazepam
 - Sedating antidepressant
 - Mirtazapine
 - Antipsychotic medications
 - Quetiapine
- Seratogenic first line
 - SSRI or SNRI:
 - Dosing similar to depression
 - Titrate weekly or biweekly
- Partial response can be augmented with:
 - Buspirone or pregabalin
- Maintain treatment at least 12 months

**** These have risks that must be weighed before considering and should only be considered after failure of treatment options ****

Psychopharmacological approaches to treating Anxiety Disorders

Huh J, Goebert D, Takeshita J, et al. Treatment of generalized anxiety disorder: a comprehensive review of the literature for psychopharmacologic alternatives to newer antidepressants and benzodiazepines. Prim Care Companion CNS Disord 2011;13.

Option #1 for Medications for GAD

- Serotonergic first line
 - SSRI or SNRI:
 - Dosing similar to depression
 - Titrate weekly or biweekly
- Partial response can be augmented with:
 - Buspirone or pregabalin
- Maintain treatment at least 12 months

Option #2

- *Other options (after first line options and CBT failure):*
 - *Benzodiazepines*
 - *Lorazepam*
 - *Sedating antidepressant*
 - *Mirtazapine*
 - *Antipsychotic medications*
 - *Quetiapine*

*** These have risks that must be weighed before considering and should only be considered after failure of treatment options*

1. Panic Disorder

1. presence of recurrent panic attacks: abrupt, intense bursts of anxiety or fear,
2. accompanied by an array of physical symptoms (e.g., rapid heart rate, dizziness, shortness of breath, sweating, nausea).



- Look out for Fear of fear= a learned avoidant response when a person avoids a situation where a panic attack is most likely, hence making that situation associated with the disorder.

- Always rule out physical reasons for possible reactions:
 - a. cardiac issues,
 - b. allergies,
 - c. Side effects to medications,
 - d. Drug use.

Epidemiology of Panic Disorder



Sex:
2:1 F:M



- Onset at late adolescence to early 20s
- Often affects ability to function at work, social, and family activities
- Up to 80% have other psychiatric disorder including depression
- Poor outcomes increased in these patients



Panic Disorder

BOX 32-1 DSM-5 Diagnostic Criteria: Panic Disorder (300.01 (F41.0))

A. Recurrent unexpected panic attacks. A panic attack is an abrupt surge of intense fear or intense discomfort that reaches a peak within minutes, and during which time four (or more) of the following symptoms occur:

Note: The abrupt surge can occur from a calm state or an anxious state.

1. Palpitations, pounding heart, or accelerated heart rate
2. Sweating
3. Trembling or shaking
4. Sensations of shortness of breath or smothering
5. Feelings of choking
6. Chest pain or discomfort
7. Nausea or abdominal distress
8. Feeling dizzy, unsteady, light-headed, or faint
9. Chills or heat sensations
10. Paresthesias (numbness or tingling sensations)
11. Derealization (feelings of unreality) or depersonalization (being detached from oneself)
12. Fear of losing control or "going crazy"
13. Fear of dying.

Note: Culture-specific symptoms (e.g., tinnitus, neck soreness, headache, uncontrollable screaming or crying) may be seen. Such symptoms should not count as one of the four required symptoms.

B. At least one of the attacks has been followed by 1 month (or more) of one or both of the following:

1. Persistent concern or worry about additional panic attacks or their consequences (e.g., losing control, having a heart attack, "going crazy").
2. A significant maladaptive change in behavior related to the attacks (e.g., behaviors designed to avoid having panic attacks, such as avoidance of exercise or unfamiliar situations).

C. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism, cardiopulmonary disorders).

D. The disturbance is not better explained by another mental disorder (e.g., the panic attacks do not occur only in response to feared social situations, as in social anxiety disorder; in response to circumscribed phobic objects or situations, as in specific phobia; in response to obsessions, as in obsessive-compulsive disorder; in response to reminders of traumatic events, as in posttraumatic stress disorder; or in response to separation from attachment figures, as in separation anxiety disorder).

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ANXIETY AND PANIC POSITIVE FEEDBACK LOOP

(BEHAVIORAL)
AVOIDANCE AND
CHRONIC ANXIETY

PANIC
(PHYSIOLOGICAL)

Tension
Hyperventilation
Increased pulse

THOUGHTS (in anticipation)

I will not be able to sleep
This is where I will panic
I can not cope
I will faint
Maybe I will have a heart attack

THOUGHTS (reactive)

This is so terrifying
I can not control this feeling
People can see me

(COGNITIVE)

INCREASED
ANXIETY

DSM-V Diagnostic Criteria

THE DIAGNOSTIC AND STATISTICAL MANUAL OF MENTAL DISORDERS , ED 5, (COPYRIGHT 2013). AMERICAN PSYCHIATRIC ASSOCIATION.

Recurrent unexpected panic attacks defined as an abrupt surge of intense fear or discomfort that reaches a peak within minutes, and during which time **four or more** of the following symptoms occur:

Palpitations, pounding heart, or accelerated heart rate

2. Sweating

3. Trembling or shaking

4. Sensations of shortness of breath or smothering

5. Feelings of choking

6. Chest pain or discomfort

7. Nausea or abdominal distress

8. Feeling dizzy, unsteady, light-headed, or faint

9. Chills or heat sensations

10. Paresthesias (numbness or tingling sensations)

11. Derealization (feelings of unreality) or depersonalization (being detached from oneself)

12. Fear of losing control or “going crazy”

13. Fear of dying.

- At least one of the attacks has been **followed by 1 month or more of one or both** of the following:

- **Persistent concern or worry about additional panic attacks** or their consequences (e.g., losing control, having a heart attack, “going crazy”).
- **A significant maladaptive change in behavior** related to the attacks (e.g., avoidance).

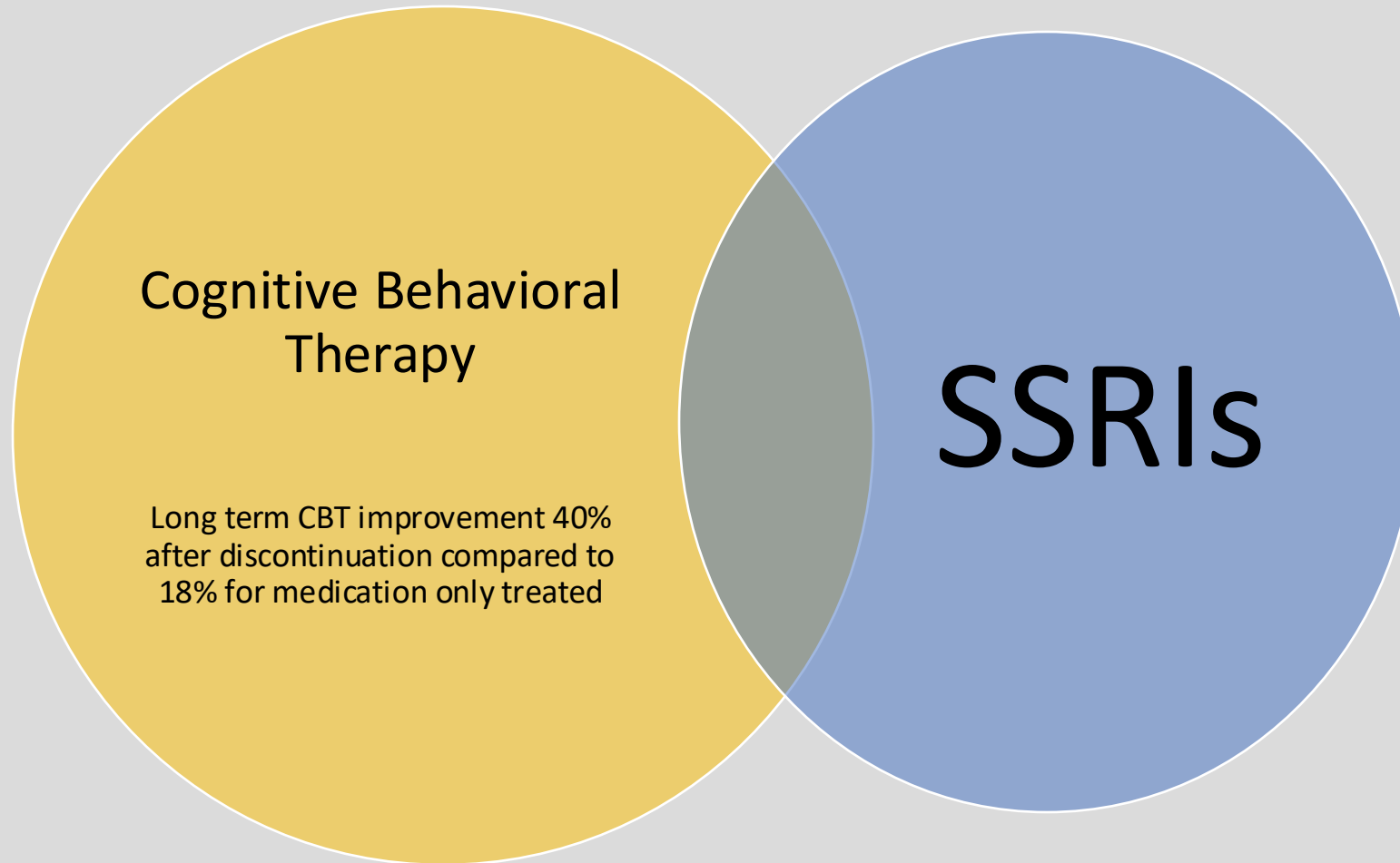
- The disturbance is **not attributable to the physiological effects of a substance or another medical condition**

- The **disturbance is not better explained by another mental disorder**

- The disturbance is not better explained by another mental disorder (e.g., the panic attacks do not occur only in response to feared social situations, as in social anxiety disorder; in response to circumscribed phobic objects or situations, as in specific phobia; in response to obsessions, as in obsessive-compulsive disorder; in response to reminders of traumatic events, as in posttraumatic stress disorder; or in response to separation from attachment figures, as in separation anxiety disorder)

Treatment for Panic Disorder

Barlow DH, Gorman JM, Shear MK, Woods SW. Cognitive-behavioral therapy, imipramine, or their combination for panic disorder: A randomized controlled trial. JAMA 2000; 283:2529.



Panic Disorder: Medication Treatment

Furukawa TA, Watanabe N, Churchill R. Combined psychotherapy plus antidepressants for panic disorder with or without agoraphobia. Cochrane Database Syst Rev 2007; :CD004364.

- First Line Psychotropic Option:
 - SSRI
 - citalopram, escitalopram, fluoxetine, paroxetine, sertraline
- Second Line Option:
 - SNRI
 - Venlafaxine (higher side effects)
 - Avoid TCAs, MAOI and benzodiazepines due to high risk of side effects and potential for abuse or overdose
- Patients with suicidality or other mental disorders should be treated with BOTH SSRIs and CBT

Social Anxiety Disorder (SAD)



Epidemiology of Social Anxiety Disorder



- Sex:
3:2 F:M
- *Equal number of M and F present for care*
- When affecting men has poorer outcomes for female patients
- Onset at late adolescence to early 20s
- Often affects ability to function at work, social, and family activities
- Up to 80% have other psychiatric disorder including depression
- Poor outcomes increased in these patients

BOX 32-3 DSM-5 Diagnostic Criteria: Social Anxiety Disorder (Social Phobia) (300.23 (F40.10))

A. Marked fear or anxiety about one or more social situations in which the individual is exposed to possible scrutiny by others. Examples include social interactions (e.g., having a conversation, meeting unfamiliar people), being observed (e.g., eating or drinking), and performing in front of others (e.g., giving a speech).

Note: In children, the anxiety must occur in peer settings and not just during interactions with adults.

B. The individual fears that he or she will act in a way or show anxiety symptoms that will be negatively evaluated (i.e., will be humiliating or embarrassing; will lead to rejection or offend others).

C. The social situations almost always provoke fear or anxiety.

Note: In children, the fear or anxiety may be expressed by crying, tantrums, freezing, clinging, shrinking, or failing to speak in social situations.

D. The social situations are avoided or endured with intense fear or anxiety.

E. The fear or anxiety is out of proportion to the actual threat posed by the social situation and to the sociocultural context.

F. The fear, anxiety, or avoidance is persistent, typically lasting for 6 months or more.

G. The fear, anxiety, or avoidance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

H. The fear, anxiety, or avoidance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

I. The fear, anxiety, or avoidance is not better explained by the symptoms of another mental disorder, such as panic disorder, body dysmorphic disorder, or autism spectrum disorder.

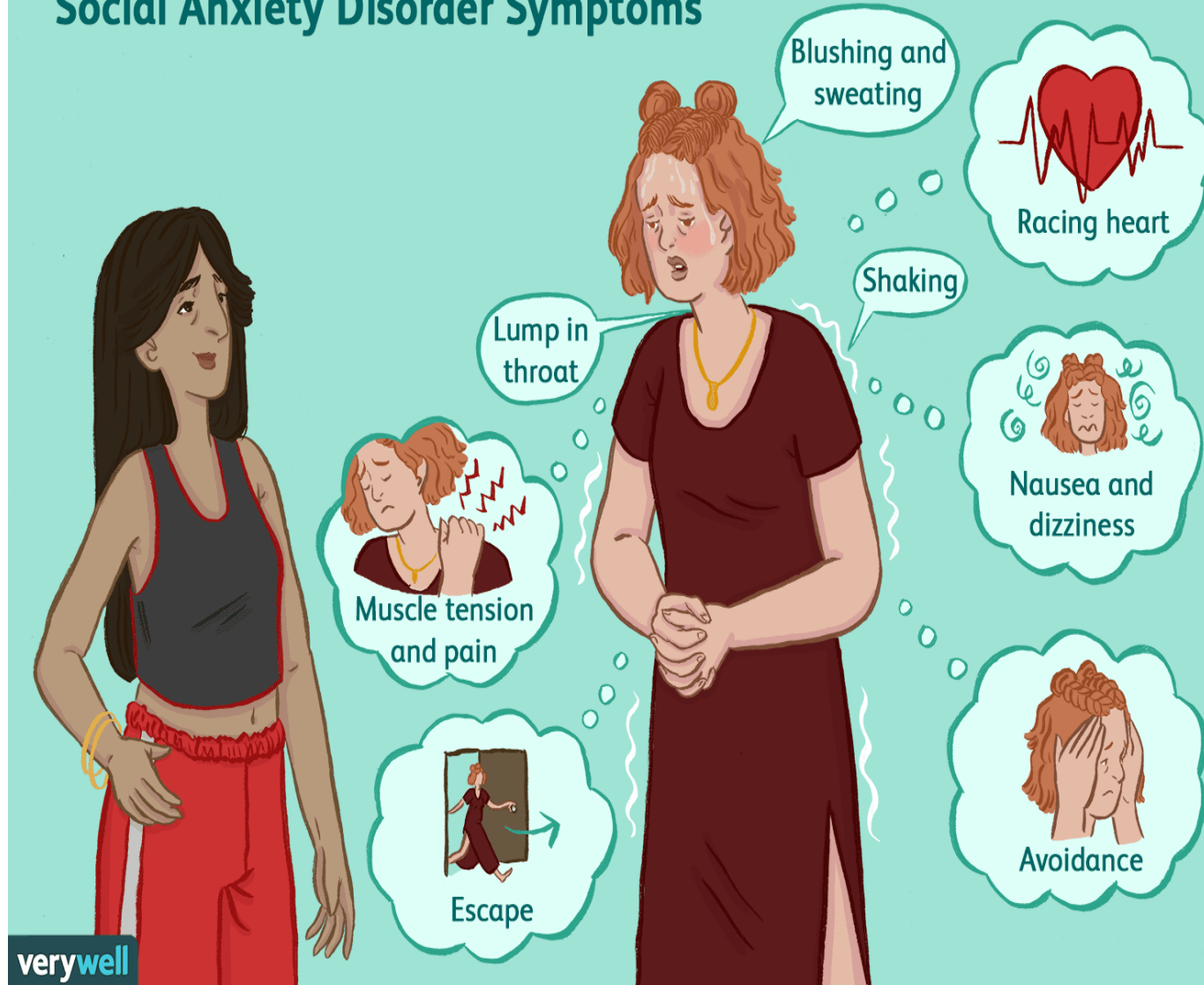
J. If another medical condition (e.g., Parkinson's disease, obesity, disfigurement from burns or injury) is present, the fear, anxiety, or avoidance is clearly unrelated or is excessive.

Specify if:

- **Performance only:** If the fear is restricted to speaking or performing in public.

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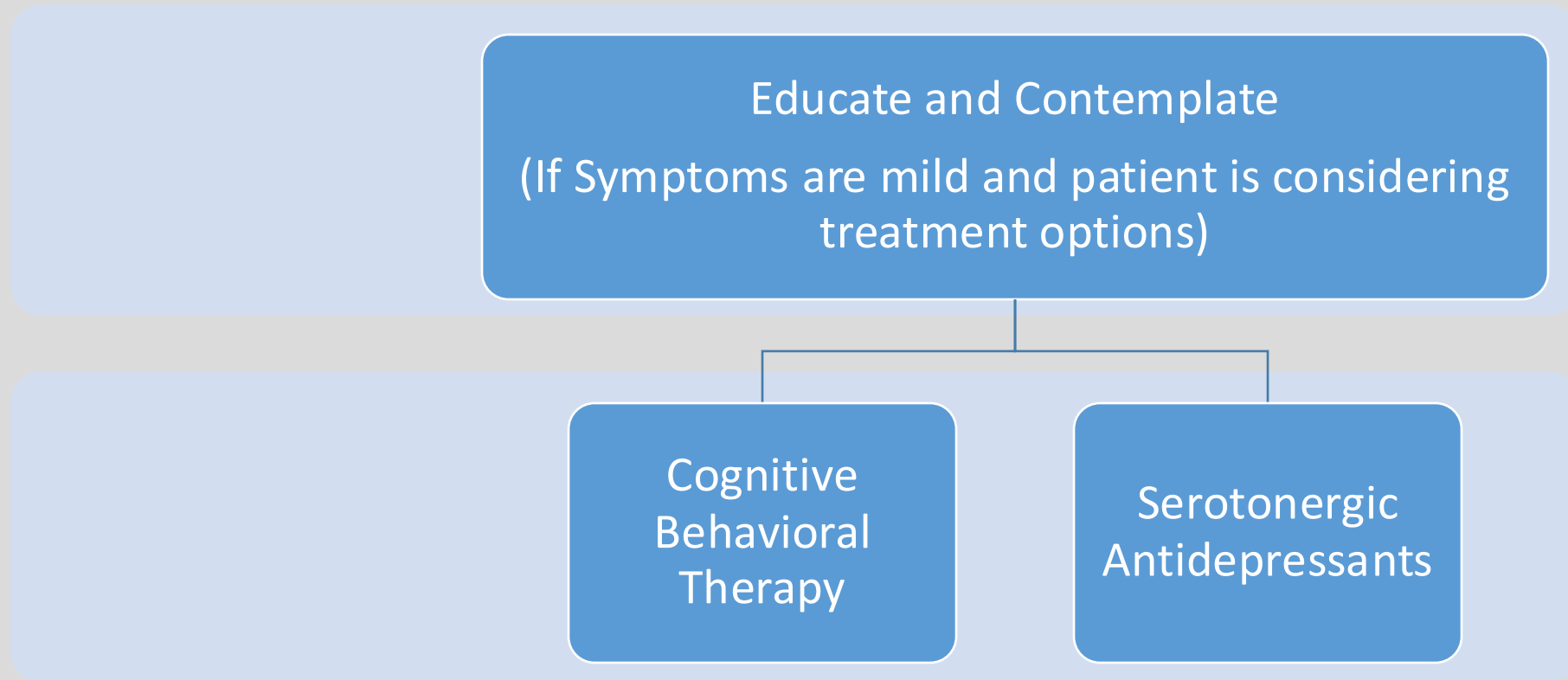
Social Anxiety Disorder Symptoms



Treatment for Social Anxiety Disorder

Stein MB, Stein DJ. Social anxiety disorder. Lancet 2008; 371:1115.

Blanco C, Heimberg RG, Schneier FR, et al. A placebo-controlled trial of phenelzine, cognitive behavioral group therapy, and their combination for social anxiety disorder. Arch Gen Psychiatry 2010; 67:286.



Medications for Social Anxiety Disorder

Pollack MH, Van Ameringen M, Simon NM, et al. A double-blind randomized controlled trial of augmentation and switch strategies for refractory social anxiety disorder. Am J Psychiatry 2014; 171:44.

- Seratogenic first line:
 - SSRI or SNRI
 - Dosing similar to depression
 - Titrate weekly or biweekly
- If treatment not effective within 10-12 weeks then change medication or include CBT
- Avoid benzodiazepines due to high risk of substance abuse

Medication for Social Anxiety Disorder, Performance Only

Steenen SA, van Wijk AJ, van der Heijden GJ, et al. Propranolol for the treatment of anxiety disorders: Systematic review and meta-analysis. J Psychopharmacol 2016; 30:128

- First Line:
 - **CBT followed by exposure therapy**
- If single or rare performance activities:
 - **Beta-blocker**
 - Propranolol
 - Benzodiazepine
 - Low dose clonazepam with caution



Cognitive Behavioral Therapy

<https://www.apa.org/ptsd-guideline/patients-and-families/cognitive-behavioral>

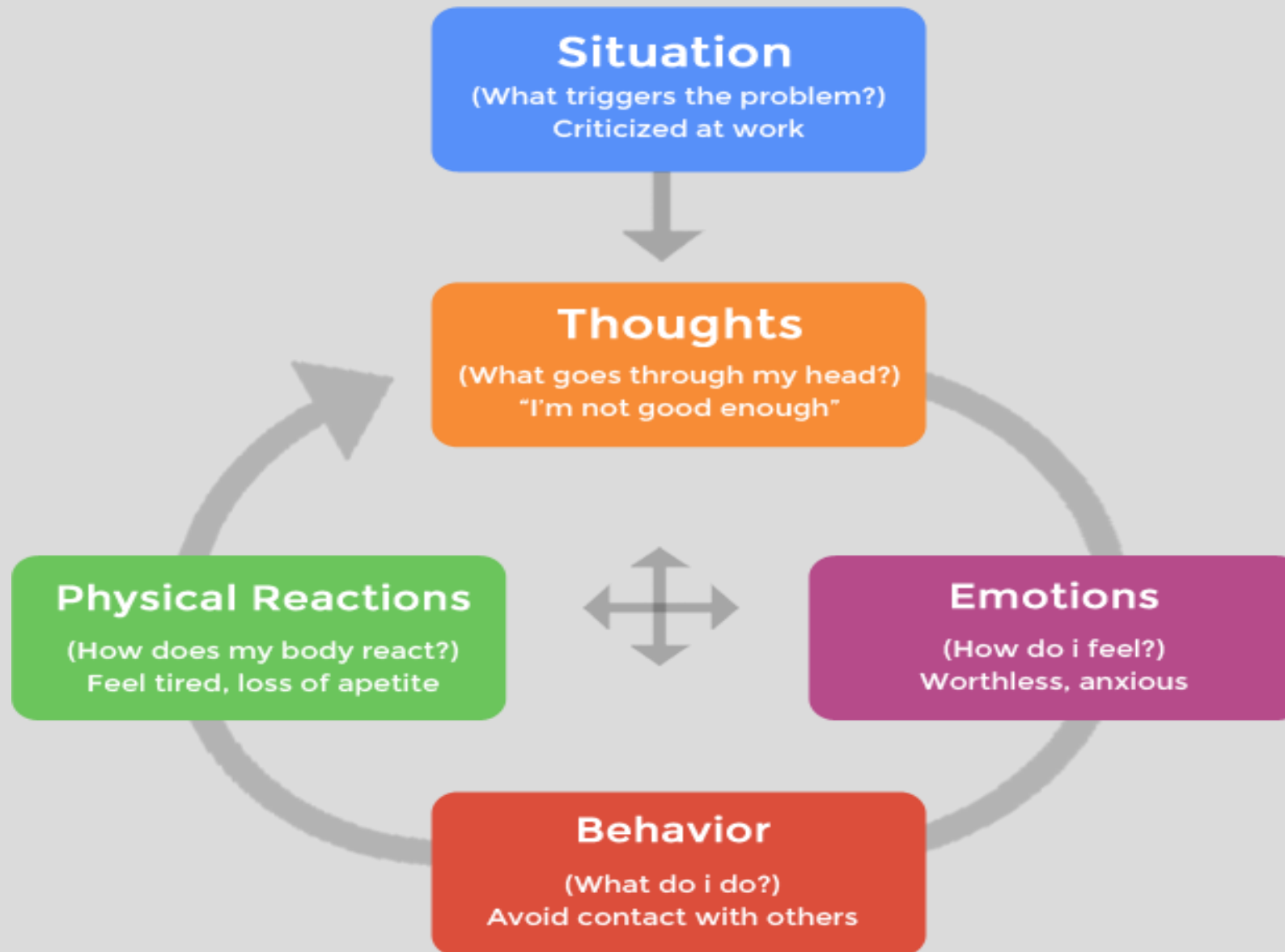
CBT is based on several core principles, including:

- Psychological problems are based, in part, on faulty or unhelpful ways of thinking.
- Psychological problems are based, in part, on learned patterns of unhelpful behavior.
- People suffering from psychological problems can learn better ways of coping with them, thereby relieving their symptoms and becoming more effective in their lives.

CBT treatment usually involves efforts to change thinking patterns. These strategies might include:

- Learning to recognize one's distortions in thinking that are creating problems, and then to reevaluate them in light of reality.
- Gaining a better understanding of the behavior and motivation of others.
- Using problem-solving skills to cope with difficult situations.
- Learning to develop a greater sense of confidence in one's own abilities.
- CBT treatment also usually involves efforts to change behavioral patterns. These strategies might include:
 - Facing one's fears instead of avoiding them.
 - Using role playing to prepare for potentially problematic interactions with others.
 - Learning to calm one's mind and relax one's body.

Cognitive Behavioral therapy



Associated disorders to be aware of...

- DSM-5 updates changed how several psychiatric disorders are categorized.
- Obsessive-Compulsive Disorder and Posttraumatic Stress Disorder are can be considered as their own disorders – although they share many similarities.

OMM technique (required)

- Please view this video about OMM techniques:

<https://www.youtube.com/watch?reload=9&v=V-9KcEZKErA>

- How many of these techniques can you administer comfortably?
- How likely are you to suggest OMM for stress?

Summary

- Anxiety Disorders are fairly common in the US population and are likely to be encountered in practice.
- Making the correct diagnosis using DSM-V criteria is critical
- Treatment typically involves CBT and/or SRI (SSRI/SNRI) options
 - rarely benzodiazepines
- Core References:
 - THE DIAGNOSTIC AND STATISTICAL MANUAL OF MENTAL DISORDERS , ED 5, (COPYRIGHT 2013). AMERICAN PSYCHIATRIC ASSOCIATION.
 - Anxiety Disorders. Massachusetts General Hospital Comprehensive Clinical Psychiatry, Chapter 32, 353-366.2016.

Resources:

1. Blanco C, Heimberg RG, Schneier FR, et al. A placebo-controlled trial of phenelzine, cognitive behavioral group therapy, and their combination for social anxiety disorder. *Arch Gen Psychiatry* 2010; 67:286.
2. Brenes GA, Danhauer SC, Lyles MF, et al. Telephone-Delivered Cognitive Behavioral Therapy and Telephone-Delivered Nondirective Supportive Therapy for Rural Older Adults With Generalized Anxiety Disorder: A Randomized Clinical Trial. *JAMA Psychiatry* 2015; 72:1012.
3. Ching, T. H. W. (2021). Culturally Attuned Behavior Therapy for Anxiety and Depression in Asian Americans: Addressing Racial Microaggressions and Deconstructing the Model Minority Myth. *Cognitive and Behavioral Practice*.
4. Chodzen, G., Hidalgo, M. A., Chen, D., & Garofalo, R. (2019). Minority Stress Factors Associated With Depression and Anxiety Among Transgender and Gender-Nonconforming Youth. *Journal of Adolescent Health*, 64(4), 467–471.
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6. Huh J, Goebert D, Takeshita J, Lu BY, Kang M. Treatment of generalized anxiety disorder: a comprehensive review of the literature for psychopharmacologic alternatives to newer antidepressants and benzodiazepines. *The primary care companion for CNS disorders*. 2011;13(2).
7. Laura J. Samander, & Jeffrey Harman. (2022). Disparities in Offered Anxiety Treatments Among Minorities. *Journal of Primary Care & Community Health*, 13.
8. Mitchell AJ, Vaze A, Rao S. Clinical diagnosis of depression in primary care: a meta-analysis. *Lancet*. 2009;374(9690):609-619.
9. National Institute for Health and Clinical Excellence, 2007. National Institute for Health and Clinical Excellence : Nice guideline 22 – Anxiety: Management of anxiety (panic disorder, with or without agoraphobia, and generalised anxiety disorder) in adults in primary, secondary and community care.
10. O'Connor E, Henninger M, Perdue LA, Coppola EL, Thomas R, Gaynes BN. Screening for Depression, Anxiety, and Suicide Risk in Adults: A Systematic Evidence Review for the U.S. Preventive Services Task Force. Evidence Synthesis No. 223. AHRQ Publication No. 22-05295-EF-1. Rockville, MD: Agency for Healthcare Research and Quality; 2022.
11. Pollack MH, Van Ameringen M, Simon NM, et al. A double-blind randomized controlled trial of augmentation and switch strategies for refractory social anxiety disorder. *Am J Psychiatry* 2014; 171:44
12. Roberge P, Normand-Lauziere F, Raymond I, et al. Generalized anxiety disorder in primary care: mental health services use and treatment adequacy. *BMC Fam Pract*. 2015;16:146.
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